

Dr Manohari Seneviratne
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Date: [DD MMM YYYY]

Patient: Dr Aruna Kulatunga
DOB: January 1962
Age/Sex: 63 years, Male
NIC/Passport: [-]
Contact: [-]

Current medications (for lab reference)

Morning:

- Empagliflozin 10 mg
- Telmisartan 80 mg (Telday)
- Hydrochlorothiazide 50 mg

Night:

- Cilnidipine 10 mg (Cilacar)
- Pregabalin 75 mg (Lyrica) hs
- Aspirin 75 mg (Ecosprin)
- Multivitamin (Swisse Men 55+)

Insulin therapy:

- Mixtard 30/70 insulin: 25 units subcutaneously twice daily, 30 minutes before breakfast and 30 minutes before dinner; rotate injection sites; carry hypoglycemia treatment and record doses and glucose in logbook.

Clinical note:

Type 2 diabetes mellitus and hypertension with burning pain and numbness in feet—suspected distal symmetric polyneuropathy. Please evaluate inflammatory, renal, metabolic, endocrine, and nutritional contributors. Report critical values by phone.

Request: Laboratory Investigations

1) Hematology

- Full Blood Count (FBC) with differential and platelets
- Erythrocyte Sedimentation Rate (ESR)
- C-reactive Protein (CRP)

2) Diabetes profile

- Fasting Plasma Glucose
- HbA1c
- Urine Albumin:Creatinine Ratio (uACR), early morning spot
- Fasting Lipid Profile

3) Renal/Metabolic

- Serum Creatinine with eGFR (CKD-EPI)

- Serum Electrolytes: Na, K, Cl, HCO₃
- Serum Calcium, Magnesium, Phosphate
- Urea (BUN)
- Urinalysis (dipstick + microscopy)
- Serum Uric Acid

4) Endocrine/Nutritional

- Thyroid Stimulating Hormone (TSH)
- Serum Vitamin B12
- Serum Folate
- 25-Hydroxy Vitamin D

5) Liver and General Chemistry

- ALT, AST, ALP, GGT, Total and Direct Bilirubin
- Serum Albumin, Total Protein

6) Neuropathy-related (as indicated)

- Serum Protein Electrophoresis (SPEP) with immunofixation if hyperglobulinemia/paraproteinemia suspected
- Fasting Plasma Homocysteine if B12/folate borderline
- Consider Nerve Conduction Studies/EMG separately (booking to be arranged if symptoms persist)

Specimen and preparation instructions

- 10-12 hours fasting for glucose and lipids; water permitted.
- Morning sample preferred for uACR.
- Continue usual medications and insulin unless otherwise advised; note last doses and any recent changes on the lab form.

Provisional diagnosis

- Painful distal symmetric polyneuropathy in a patient with diabetes and hypertension—rule out inflammatory, renal, endocrine, and nutritional causes.

Signature: _____

Dr Manohari Seneviratne

Date/Time: _____